ZIMBABWE YOUTH INNOVATION HACKLAB 2026

A safe space to talk.

A path to real support.



TALK • LEARN • CONNECT

Category 1 · Holistic Youth Health & Well-being

01 / THE CHALLENGE

The hardest part can be asking for help.

We are testing three barriers that can stop a young person before they reach support.

01

Stigma

Fear of being judged can make opening up feel unsafe.

02

Uncertainty

People may not know which service is credible, affordable or approachable.

03

The first contact

Even finding the words to ask for a conversation can feel difficult.

Problem hypothesis — primary research and clinical consultation still to be completed.

02 / ONE CONNECTED FIRST STEP

Three simple spaces. One clear next step.

Users are never forced to chat with AI before exploring people and services.

01 TALK

A softer place to start

Optional guided reflection and grounding. Scripted demo today.

02 LEARN

Knowledge without noise

Original awareness content and links to independent sources.

03 CONNECT

A path to real people

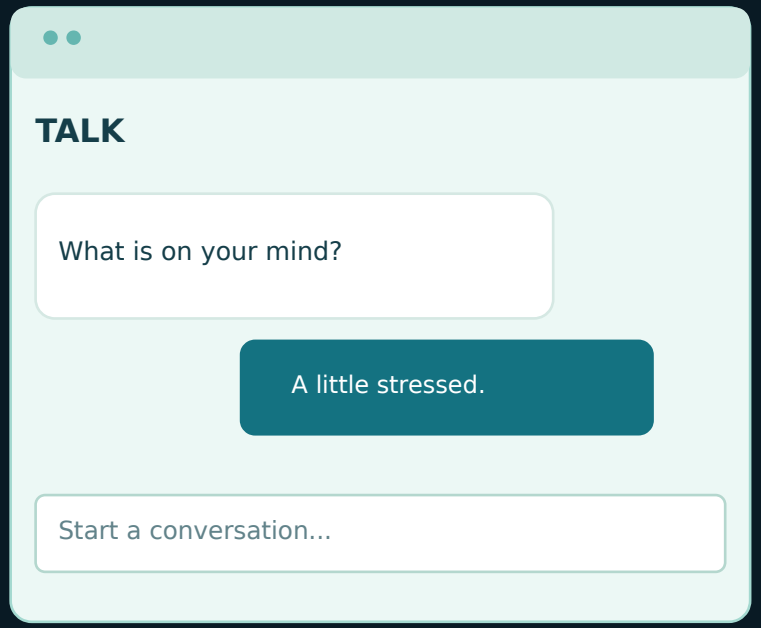
Choose care preferences, prepare a first message and explore services.

Choice and consent connect these spaces; no private chat is automatically shared.

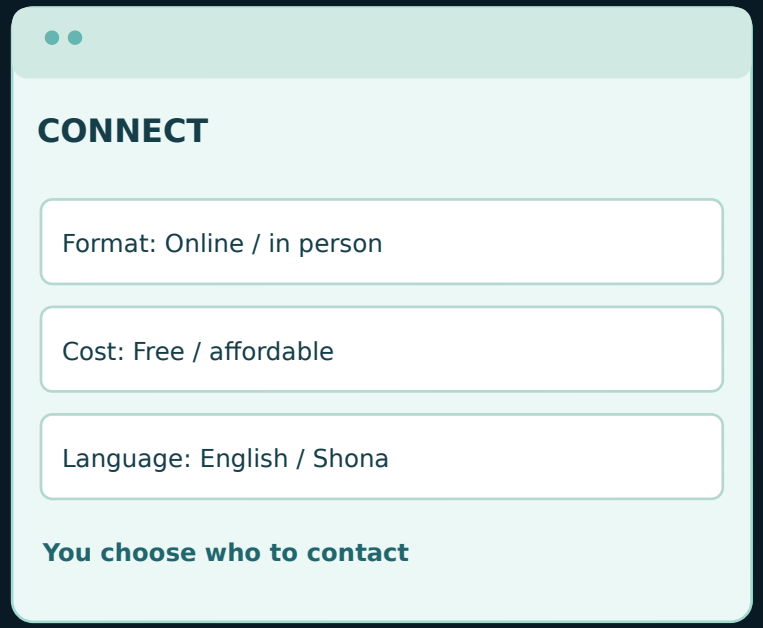
03 / WHAT WE HAVE ACTUALLY BUILT

Working prototype, not just a concept.

These are simplified illustrations of features available in the live demo; not clinical functionality.







OPEN THE LIVE DEMO ↗

zim-safe-minds.vercel.app

04 / OUR DISTINCTION

Not an “AI therapist.” A bridge to human care.

Existing providers deliver meaningful services. We want to make approaching care less intimidating.

- **Choice** — A user can skip the conversation entirely and go straight to care information.
- **Privacy-first** — No automatic transcript forwarding. A first-contact message is controlled by the user.
- **Human accountability** — Verified provider introductions and live appointments require qualified partners and approval.

Friendship Bench is an existing independent service, not a signed project partner.

05 / DESIGNED FOR ACCESS

Built for young people, on their terms.

Initial research focus: consenting adults aged 18–35 in Zimbabwe.

LOW-DATA

Accessible on entry-level Android phones; low-bandwidth design is a pilot goal.

LANGUAGE

English, Shona and Ndebele preferences in the current navigator; clinical localization later.

INCLUSION

Gender-aware, disability-aware and affordability-aware research and testing.

PILOT GOALS, NOT CURRENT RESULTS

Up to 50 consenting adults · Goal: 2 verified providers

06 / FEASIBILITY

A careful 12-month pilot, not a rushed clinic.

Clinical oversight, privacy review and independent provider verification gate any real referral features.

MONTHS 1–2	**Listen & learn**	Youth interviews and professional consultations
MONTHS 3–4	**Design safely**	Clinical safeguarding, privacy and accessibility review
MONTHS 5–8	**Build & pilot**	Verify partners; controlled adult pilot only after approval
MONTHS 9–12	**Evaluate & refine**	Report outcomes, costs, harms, and scale/pivot decision

No live AI triage, diagnoses, or professional bookings are claimed today.

07 / THE ASK

US$5,000. Applied carefully.

Illustrative spending allocation across research, safeguarding, accessible engineering and evaluation.



TOTAL REQUEST **$5,000**

08 / TEAM-LED INNOVATION

A working prototype.
A team ready to shape what’s next.

TEAM LEAD
To be confirmed by the team

TECHNICAL LEAD
Andrew Hama Mutamiri

RESEARCH & WELLBEING
Member to choose role

INCLUSION / OPERATIONS
Member to choose role

Talk. Learn. Connect. → Real care.

APPENDIX / DEMO SEQUENCE

Three minutes. One believable story.

The host should have the live URL loaded, plus this deck available offline.

00:00–00:35	**TALK**	Show original Afterglow visuals; open consent-gated scripted chat.
00:35–01:15	**LEARN**	Search a topic, open an awareness card, show the source.
01:15–02:30	**CONNECT**	Select preferences and copy a sample first-contact inquiry.
02:30–03:00	**CLOSE**	Show independent service information; recap ethics and grant use.

APPENDIX / DIFFICULT QUESTIONS

Questions we should welcome.

Is this ChatGPT for therapy?	No: the current conversation is scripted; any future AI requires clinical oversight and safety validation.
Why not just use Friendship Bench?	It is an independent existing service. We will test the first-contact navigation gap without duplicating care.
Do you have signed provider partners?	Not yet. Credential checks, agreements and approvals are explicit pilot milestones.
What does success look like?	Real opt-in contact with appropriate care; include drop-offs and safety incidents, not chatbot minutes.